

CLINICAL TRIAL PROTOCOL

A Phase IIa, Multi-Center, Randomized, Double-Blind, Placebo-Controlled Study to Evaluate the Efficacy and Safety of UX-8822 in Patients with Moderate-to-Severe Chronic Plaque Psoriasis

Investigational New Drug (IND) Reference Number: 144,902

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1. Background & Rationale

Compound UX-8822 is a novel, highly selective, small-molecule allosteric inhibitor of tyrosine kinase 2 (TYK2). By suppressing TYK2 activation pathways, the drug explicitly inhibits downstream signaling networks mediated by interleukin-23 (IL-23), interleukin-12 (IL-12), and type I interferons (IFN). Crucially, at therapeutic concentrations, UX-8822 exhibits more than 50-fold selectivity for TYK2 relative to Janus kinase 1 (JAK1), JAK2, and JAK3, minimizing the risk of off-target myelosuppression and lipid profile anomalies.

Adverse Event Operational Threshold

Any localized or systemic event demonstrating an absolute neutrophil count drop below **1,000 cells/ μ L** dictates immediate structural treatment suspension and protocol unblinding evaluation.

2. Trial Design & Subject Stratification

This is a 16-week, multi-center, parallel-group study targeting an enrollment of 180 eligible subjects randomized in a strict 1:1:1 ratio into three discrete evaluation cross-arms:

- **Cohort Alpha (A):** UX-8822 15 mg administered orally once daily (QD).
- **Cohort Beta (B):** UX-8822 30 mg administered orally once daily (QD).
- **Cohort Gamma (Γ):** Matching placebo administered orally once daily (QD).

2.1 Core Screening Inclusions and Exclusions

Eligible candidates must present a confirmed clinical diagnosis of stable plaque psoriasis for ≥ 6 months prior to the initial screening timeline. The operational diagnostic metrics required for baseline validation are defined systematically in the matrix below:

Clinical Parameter Indicator	Minimum Inclusion Cutoff	Absolute Exclusion Threshold	Assessment Method
PASI Score	Score ≥ 12.0	Score < 12.0 at Screening	Blinded Clinical Evaluator
Body Surface Area (BSA)	Involvement $\geq 10\%$	Localized variants only ($< 10\%$)	Rule of Nines Mapping
sPGA Rating	Grade ≥ 3 (Moderate)	Grades 0, 1, or 2	Static 5-Point Scale
Active Biological Washout	> 12 weeks for anti-IL17/23	Concurrent systemic therapy	Patient History Log Verification

3. Primary and Secondary Endpoint Frameworks

3.1 Primary Efficacy Criterion

The primary outcome metric is the proportion of subjects achieving a Psoriasis Area and Severity Index 75 (PASI 75) response—defined as an absolute 75% or greater reduction in the cumulative PASI score from the baseline reference profile—at the culmination of Week 12.

3.2 Key Secondary Criteria

Secondary metrics designed to evaluate prolonged kinetic efficacy trajectories across the cohorts include:

1. The proportion of subjects achieving a static Physician's Global Assessment (sPGA) score of 0 (Clear) or 1 (Almost Clear) with at least a 2-point improvement relative to baseline at Week 12 and Week 16.
2. Logarithmic reduction curve patterns of serum C-reactive protein (CRP) levels evaluated from blood draws gathered during scheduled diagnostic intervals (Weeks 2, 4, 8, 12, and 16).
3. Total percentage shift in the Dermatology Life Quality Index (DLQI) metrics profile.

4. Investigational Product Management & Dispensing Log

UX-8822 and matching placebos are supplied as round, film-coated tablets packaged in high-density polyethylene (HDPE) bottles containing a built-in child-resistant closure configuration. All clinical supply configurations must be stored within a secure, temperature-monitored, restricted-access pharmaceutical vault operating consistently under the following specified environmental state:

Controlled Room Temperature Range: 20°C to 25°C (68°F to 77°F)

Excursions fluctuating temporarily down to 15°C or spiking briefly up to 30°C (59°F to 86°F) are permitted in compliance with standard USP regulations, provided the deviation period does not accumulate beyond 24 consecutive runtime hours before corrective facility remediation steps are completed.